

CLINICAL PRACTICE GUIDELINE	
Women's and Perinatal Mental Health Referral and Management	
Scope (Staff):	Health professionals caring for women and families in the perinatal period, including mental health staff working in the Mother and Baby Unit and the Department of Psychological Medicine
Scope (Area):	Women's and Perinatal Mental Health
This document should be read in conjunction with the Disclaimer .	

Contents

1. Aim	2
2. Mental Health Services at WNHS	3
2.1 Mother and Baby Unit.....	3
2.2 Department of Psychological Medicine.....	3
3. Referral.....	3
3.1 Mother and Baby Unit.....	3
3.2 Department of Psychological Medicine.....	4
4. Screening.....	5
5. Triage	7
6. Risk Assessment	8
6.1 Risk Assessment for inpatients.....	8
6.2 Risk Assessment for outpatients	8
6.3 Risk assessment for infants and mandatory reporting requirements	8
7. WNHS Mental Health Model of Care.....	9
Overview.....	10
8. Mental health assessment	10
8.1 Assessment of all clients	10
8.2 Assessment of clients in the perinatal period	11
Assessment of the mother	12
Assessment of the mother-(fetal) infant relationship	13
8.3 Assessment of women referred from gynaecology and gynae-oncology	13
9. Treatment and specific interventions.....	15
9.1 Providing information and seeking informed consent.....	15
9.2 Psychological treatment.....	16
9.3 Pharmacological treatment.....	19

9.4	Electroconvulsive Therapy (ECT)	29
9.5	Complementary therapies	29
9.6	Perinatal Mental Health Plans	29
10.	Discharge Planning	30
11.	Telehealth	30
12.	Risk Management/Safety and Security of Staff	31
	Principles in responding to requests for post incident trauma debriefing	31
13.	Documentation	32

1. Aim

This guideline provides direction for mental health clinicians working in the Women and Newborn Health Service (WNHS) to support women and their families accessing mental health services.

This guideline:

- Highlights key concerns and recommendations in the care and treatment of women's and perinatal mental health.
- Outlines structured evidence-based models of care for provision of psychological and pharmacological treatment for women's and perinatal mental health.
- Provides staff with clear pathways and guidance, specific to WNHS, for referral, screening, assessment, and treatment of mental health conditions.
- Ensures that when medication and psychological treatments are used to support women experiencing mental illness, they are prescribed and/or administered responsibly.
- Emphasises recovery-oriented practice which builds on patients' individual strengths; emphasising hope, social inclusion, community participation, personal goal setting, and self-management.
- Prioritises the treatment of mental health conditions using a consistent mother-infant approach that considers the family as a holistic unit.
- Promotes high quality patient care in accordance with the [Charter of Mental Health Care Principles](#), and consistent with the [Mental Health Act 2014 \(MHA 2014\)](#) and the [Chief Psychiatrist's Standards for Clinical Care](#).

2. Mental Health Services at WNHS

2.1 Mother and Baby Unit

The [Mother and Baby Unit](#) (MBU) is an authorised in-patient treatment centre for acute psychiatric conditions in the perinatal period, located at the King Edward Memorial Hospital (KEMH). Women and their babies may be admitted to the inpatient program if they have significant mental health problems following the birth of a baby and up to 12 months postpartum. Admission can also be offered to women who are experiencing mental health problems in the antenatal period requiring inpatient treatment.

2.2 Department of Psychological Medicine

The [Department of Psychological Medicine is an outpatient and consultation-liaison service that offers](#) assessment and treatment of mental health for current patients of Obstetrics, Gynaecology and Neonates at KEMH. Psychological Medicine is a multidisciplinary team with expertise in treating women's and perinatal mental health.

3. Referral

3.1 Mother and Baby Unit

Any medical practitioner or mental health professional can make a referral to the MBU by completing the [referral form](#).

Internal referrals from Psychological Medicine staff can be made directly to the MBU, requiring a brief documented summary. The on call psychiatrist is responsible for accepting referrals out of hours.

The following eligibility criteria apply for referral to the Mother and Baby Unit:

- Pregnant women (28 weeks gestation and over or earlier if deemed clinically appropriate) with moderate to severe mental health problems who are obstetrically stable, physically well, and agree to admission to the MBU.
- Women with moderate to severe mental health problems, whose baby or babies are non-toddling, are aged up to 12 months, and there is a major impact on the women's level of functioning and/or ability to parent.
- Women/babies with infectious disease and/or illness must be symptom free prior to admission consistent with the WNHS Infection Prevention and Management Manual - [Transmission Based Precautions](#)

Women with substance and alcohol dependencies will only be considered eligible for referral following completion of withdrawal and detoxification.

Women who require a highly secure or intensive nursing environment, are not eligible for referral until deemed able to be managed safely on MBU.

MBU staff should refer to the MBU Procedure Manual for further details about eligibility and exclusion criteria for referral, referral process, and referral list management.

3.2 Department of Psychological Medicine

Clinicians working at WNHS can make a referral to Psychological Medicine using the [E-Referral process](#). Health professionals external to WNHS can make a referral of eligible women for the service to the Psychological Medicine Department using the [referral form](#). [Patients are also able to self-refer](#).

The following eligibility criteria apply for referral to Psychological Medicine:

- Women with mental health concerns and who are current patients of obstetrics (including all outpatient clinics and services) or have admitted neonates at KEMH or who received care at KEMH for their most recent pregnancy and are within 12 months postpartum.
- Women who have been discharged from the Mother and Baby Unit and for whom follow-up at the Department of Psychological Medicine is deemed appropriate.
- Women with severe mental illness who require antenatal management by the state-wide multidisciplinary [Childbirth and Mental Illness Clinic \(CAMI\)](#). This service also includes a single session pre-conception consult with a psychiatrist for women with a severe mental illness.

Criteria for referral to the CAMI Clinic include:

- Low prevalence psychotic disorders (including schizophrenia and schizoaffective disorders and other psychotic disorders)
- Bipolar Affective Disorders (mainly BPAD type 1) (bipolar I is a low prevalence disorder also)
- Women who are pregnant with a Body Mass Index of below 15 and Anorexia Nervosa who require antenatal management by the Maternal Fetal Medicine Gold Antenatal Clinic - see [Eating Disorders in Pregnancy Guideline](#)
- For gynaecological and gynae-oncology referrals, where a mental health co-morbidity exists and requires assessment and short-term management with care being provided by the KEMH gynae/gynae-oncology treating team, use the [E-Referral Forms](#).

The referral form includes criteria to determine level of urgency:

Emergency referrals	This is for inpatients only as the outpatient service does not provide a crisis or emergency assessment service. For inpatients only at KEMH contact triage nurse (81521) <i>or</i> after-hours registrar or psychiatrist via KEMH switchboard (e.g. patient has acute psychosis or is suicidal).
Moderately urgent referrals	Contact triage nurse (81521) <i>and</i> e-referral/email/fax referral form (e.g. patient is stable, with moderate to severe impact on function). An appointment will be offered within 1-2 weeks
Routine referrals	E-referral/email/fax/mail referral form – patient will be contacted for an outpatient appointment (e.g. patient is stable, with no psychiatric symptoms requiring urgent treatment). This will be offered within 4-6 weeks

Psychological Medicine staff should refer to the Psychological Medicine Procedure Manual for further details about eligibility and exclusion criteria for referral, referral process, and referral list management.

While criteria for perinatal women includes from conception to 12 months postpartum determination of whether the presenting concerns require a perinatal mental health service or referral to a community service is still made.

4. Screening

Screening is used to inform the perinatal mental health referral process, and is undertaken by midwifery staff in antenatal clinics, paediatric nursing staff in neonatal settings, and visiting community midwives. There is no routine mental health screening in place for referral of women attending gynae/gynae-oncology.

As per recommendations in the Centre of Perinatal Excellence (COPE) [Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline](#) (p.23 – p. 41), screening uses the [Edinburgh Postnatal Depression Scale \(EPDS\)](#) and the [Antenatal Risk Questionnaire](#) (ANRQ) antenatal screening tools according to the following schedule:

Edinburgh Postnatal Depression Scale	In the antenatal period at 12 weeks or first visit; and at least once later in pregnancy, between 28 – 32 weeks
	In the postnatal period at 6 – 12 weeks, and at least once in the first year
	Any other time (opportunistic screening) in the first perinatal year and during psychological treatment
Antenatal Risk Questionnaire	At any time the EPDS is administered antenatally, or as indicated.

Clinical assessment must consider women's presenting concerns as well as the ANRQ in conjunction with the EPDS and the anxiety subscale of the EPDS.

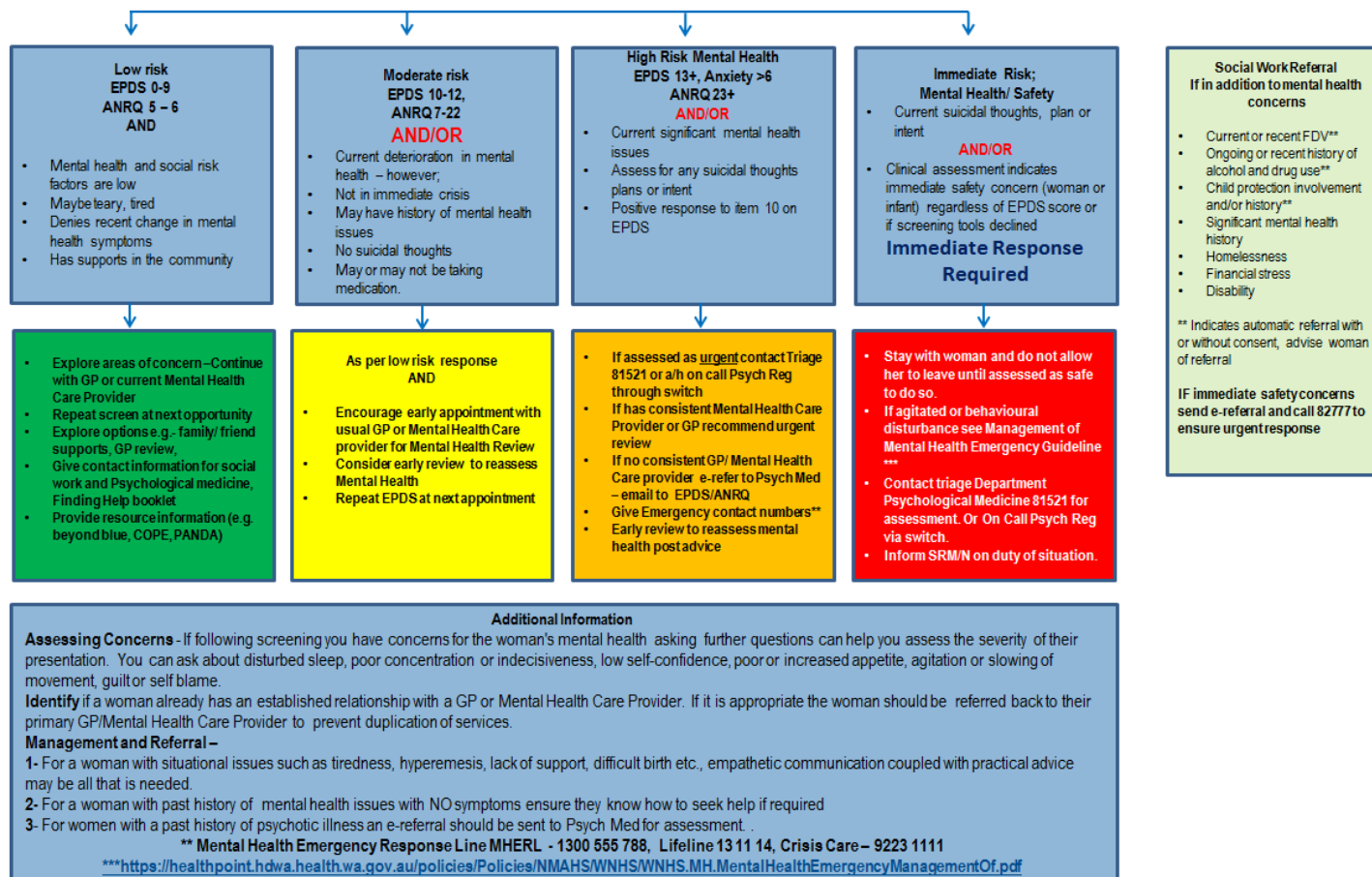
For information on responding to mental health emergencies and the management of Acute health-related Behavioural Disturbance (ABD), see the [Management of Mental Health](#) Emergency guideline.



5. Triage

Perinatal Mental Health Referral pathways see pathways diagram below.

KEMH Perinatal Mental Health Referral Pathways



Psychological Medicine staff should refer to the Psychological Medicine Procedure Manual for further details about triage process.



6. Risk Assessment

Mental health risk assessment and risk management processes – designed to assess, minimise, and manage risks in relation to risk to self, to others, and from others – are mandatory at WNHS, as defined by the [MHA 2014](#).

The processes and criteria for risk assessment and management are outlined in the Chief Psychiatrist's Standards for Clinical Care: [Risk Assessment and Management Standard](#).

Use the following guidance to undertake risk assessment:

6.1 Risk Assessment for inpatients

Assess patients using the State-wide Standardised Clinical Documentation (SSCD) for Mental Health Services [Risk Assessment and Management Plan](#).

For assessment and management of Acute Behavioural Disturbance refer to Flowchart 1 Acute health-related Behavioural Disturbance (ABD) – Inpatients contained within the [Management of Mental Health Emergency Guideline](#) (p.2).

6.2 Risk Assessment for outpatients

Assess patients using the State-wide Standardised Clinical Documentation (SSCD) for Mental Health Services [Risk Assessment and Management Plan](#).

For assessment and management of Acute Behavioural Disturbance refer to Flowchart 2 Acute health-related Behavioural Disturbance (ABD) – Outpatients contained within the [Management of Mental Health Emergency Guideline](#) (p.3).

6.3 Risk assessment for infants and mandatory reporting requirements

The [Children and Community Services Act 2004](#) is the Act concerning child protection.

When undertaking a risk assessment for infants and if considering mandatory reporting or discussion with Department for Communities, Child Protection and Family Support Services (CPFS) it is required to ensure co-ordination at KEMH that these cases be discussed with Social Work. If a referral to Social Work is not already in place this is required. Specific guidance for social workers assessing child protection risks to newborn and unborn children can be found in the [Child Protection - Social Work](#) Clinical Practice guideline. This guideline outlines social work referral and assessment for child protection risk and referral process to CPFS if deemed appropriate.

All WA health staff have a professional obligation to take action on behalf of a child that they believe is being, or is likely to be, abused or neglected and are legally obliged to comply with the mandatory reporting of child sexual abuse legislation.

Mandatory reporting of sexual abuse is required by medical, nursing and midwifery staff.

For further guidance on reporting requirements, including mandatory reports, staff should refer to the WNHS [Child Protection and Child Sexual Abuse \(Mandatory Reporting\) Policy](#).

The WA Department of Health [Guidelines for Protecting Children 2015](#) provides information and procedural guidance to support health system staff promote and safeguard the wellbeing of children when they are at risk of, or have been harmed through abuse and/or neglect. Page 2 of the [Guidelines for Protecting Children 2015](#) summarises a 4 step process in the identification and response to possible child abuse and neglect.

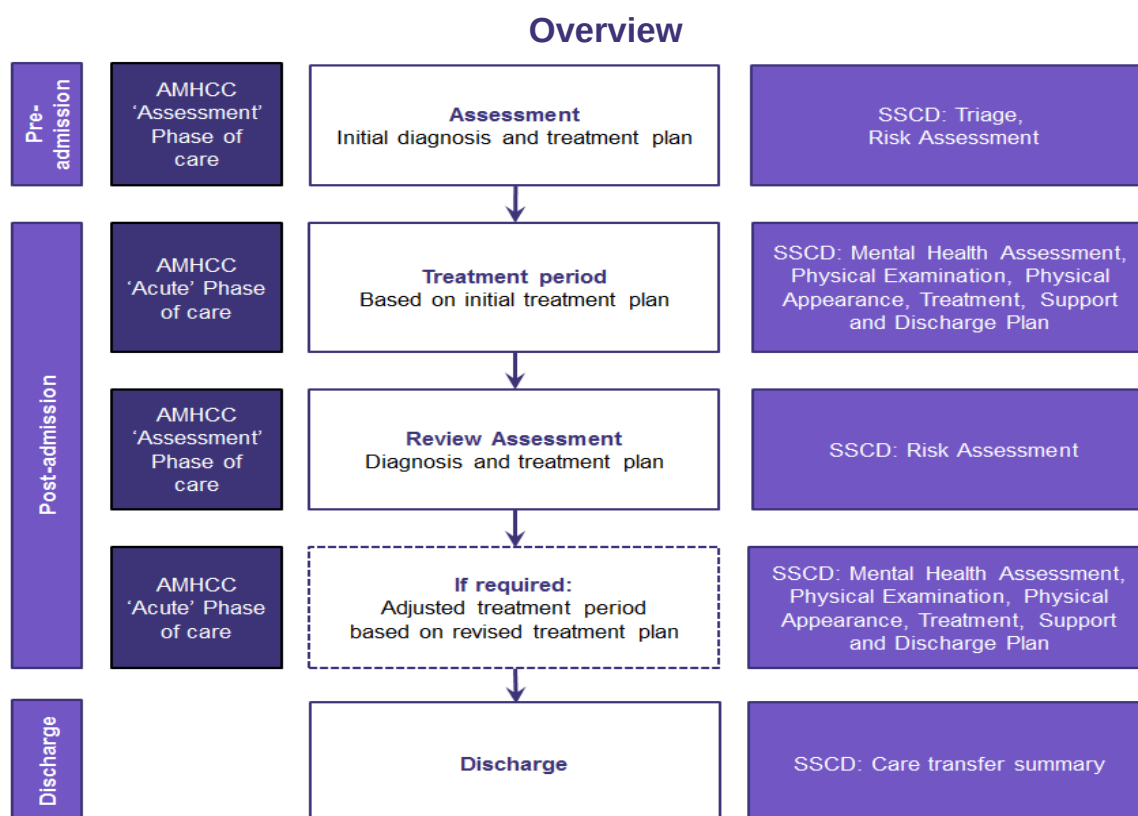
7. WNHS Mental Health Model of Care

The model of care for mental health treatment in WNHS is supported by a formal mental health assessment process and the implementation of a documented mental health treatment plan; with ongoing review and adjustment if required.

Developing clear plans for pharmacological and psychological intervention and duration of treatment – that are evidence supported– ensures patient compliance with treatment, a high quality of service, and improved patient outcomes.

Mental health care is offered at WNHS on a ‘needs basis’ to address mental health concerns relating to patients’ gynaecological and/or obstetric medical conditions and mothers of admitted neonates as well as inpatients of the MBU. WNHS mainly consists of [Australian Mental Health Care Classification](#) ‘Assessment only’ and ‘Acute’ phases of care as depicted in the diagram below.

- Assessment only - obtain information to determine the intervention/treatment needs and to arrange for this to occur such as the KEMH Consultation-Liaison service
- Acute - short term reduction in severity of symptoms associated with recent onset or exacerbation



8. Mental health assessment

8.1 Assessment of all clients

Mental health assessment evaluates the individualised needs of a client and their family or carer, in order to address mental health concerns through mental health treatment, care, and recovery planning.

Respecting diversity and ensuring inclusion are central to mental health services provided at WNHS. This includes understanding the importance of spiritual and cultural needs of Culturally and Linguistically Diverse and Aboriginal and Torres Strait Islander women and their families and ensuring there is a collaborative approach when providing care. Staff should refer to the [Charter of Mental Health Care Principles](#), specifically principle 7, and the [DOH MP 0065/17 Aboriginal Cultural eLearning Policy](#). Furthermore, there needs to be consideration of the needs of LGBTIQ+ patients and their families in all aspects of service delivery. Staff should refer to the [Western Australia Lesbian, Gay, Bisexual, Transgender, Intersex Health Strategy 2019-2024](#) document for additional information.

The processes and criteria for mental health assessment are outlined in the Chief Psychiatrist's [Standards for Clinical Care: Assessment Standard](#).

Mental health clinicians should assess all patients receiving mental health treatment in accordance with the Chief Psychiatrist's Standards using the SSCD for Mental Health Services [Mental Health Assessment](#) form.

For all patients under Psychological Medicine and MBU the National Outcome Casemix Collection (NOCC) outcome measures are required at time of admission, review and discharge. For further information regarding NOCC measures refer to [Adult NOCC Rater Clinical Utility Training Manual](#) and [Mental Health National Outcomes and Casemix Collection - Technical Specification of State and Territory Reporting Requirements Version 2.02](#).

Additional clinical measures can be utilised to (i) supplement screening, (ii) clarify diagnosis or (iii) as a measurement of change pre and post an intervention. Staff need to consider the strength of the psychometric properties of any measures used, and if using in perinatal women whether the measure has been evaluated during the perinatal period. Additionally, for non-mental health measures (e.g. mother-infant relationship measures) consider for what purpose the measures have been validated for use and furthermore their applicability in women with mental disorders. If utilising copyright instruments staff must ensure compliance with copyright. Clinicians should also apply and consider principles of trauma informed care.

Following assessment (or changes in management of the patient) a letter should be forwarded to the referrer, the patient's General Practitioner, the Child Health Nurse where appropriate or to other community agencies, and the patient (unless contraindicated) outlining the assessment and recommendations for management.

8.2 Assessment of clients in the perinatal period

In addition to the standard mental health assessment, patients in the perinatal period receiving treatment from Psychological Medicine and in the Mother Baby Unit should be assessed according to the following three domains:¹

- The mother
- The infant; or relevant criteria related to the fetus for antenatal assessments
- The mother-infant relationship

While this is best practice it is not always possible or appropriate to assess the mother-infant relationship or the infant within clinical care. Where possible and appropriate, involve the partner and/or carer in the assessment process.

These are guidelines for assessment and will need to be adapted or modified for specific presentations or circumstances.

¹ *Guidelines for the Assessment of Perinatal Psychiatric Disorders – Mercy Hospital for Women*

Assessment of the mother

Explore the following additional criteria when undertaking SSCD assessment of the mother:

Reason for referral, <i>standard adult mental health assessment</i> , risk assessment (suicide and infanticide), previous postpartum mental illnesses
Relevant details regarding previous obstetric or gynaecological women's health conditions including menarche and menstrual disorders
Ideas/beliefs about pregnancy and baby prior to conception, and any change/s during pregnancy or since delivery
Any history of infertility, termination of pregnancy or miscarriage/stillbirth
Experience of pregnancy and delivery (is/was the pregnancy planned and wanted, complications, distress, traumatic events)
A detailed family relational history including mother (and father if possible): past and present relationship with parents, parental warmth, punishment, conflict, relationship with peers, school experiences, and current relationships including family domestic violence
The mother's early childhood history and the impact on current presentation; and the mother's experiences of being parented
Cognitions, perceptions, behaviours that impact on infant's safety and development
Quality of marital/partner/family relationships; psychosocial supports/isolation/distress in pregnancy

Assessment of the infant; or relevant criteria related to the fetus for antenatal assessments

Explore the following criteria when undertaking assessment of the infant or fetus²:

Details of pregnancy and delivery: any complications/concerns, length of gestation, neonatal history, and mother's medical history since birth
Growth and developmental history: growth in utero, physical development since birth (weight gain, length and head circumference), acquisition of appropriate developmental milestones (social, motor, language)
Self-regulation: capacity to regulate affect, any settling difficulties, distress, capacity to regulate sleep/feeding and capacity to adapt to new situations – including

² If concerns are identified these are to be referred to appropriate services for further assessment including GP, Child Health Nurse, Paediatrics, CAMHS. A useful resource to aid screening is the parent held Child Health "All About Me" book (commonly referred to as "The Purple Book").

developmentally-appropriate responses to changes in their environment (new sights/sounds etc.), attention span, frustration tolerance, and activity levels
Feeding: any concerns in the pattern or capacity to feed (e.g. failure to thrive); enjoyment of the mother and infant in the feeding process
Sleeping: patterns of infant's sleep, changes over time, and parental expectations of sleep patterns
Separation anxiety: (appropriate wariness, over-anxious or lack of wariness), role of transitional objects for the infant or other soothing behaviours
Behavioural problems
Language development: including 'proto-conversations'
Capacity for developmentally appropriate play.
If further Developmental Screening or Assessment required, consider either maternal report measure (e.g. Ages and Stages Questionnaire) or neuropsychological administered scale (eg Bayley Scales of Infant Development)

Note: For physical assessment and care requirements of the baby when admitted to MBU, see MBU Procedure Manual.

Assessment of the mother-(fetal) infant relationship

Explore when undertaking assessment of the dyadic relationship including in pregnancy. Consider (i) the quality of the relationship, such as attachment, (ii) parenting stresses and supports and (iii) confidence in relevant parenting and interaction skills. Where possible in the postpartum use observation and self-report; observe in different situations – ask the mother to play with her baby as she normally would.

Consider the use of maternal self-report and observational assessment measures when clinically indicated.

8.3 Assessment of women referred from gynaecology and gynaecology

A standard mental health assessment with relevant consideration of details relevant to gynaecological or gynaecology presentation.

In addition, consideration of:

- The impact of the gynaecological condition and related symptoms/factors such as pain, bowel and bladder dysfunction, prolapse, sexual dysfunction etc on the individual's functioning and quality of life.
- Assessment of the patient's beliefs about their gynaecological condition, self-efficacy and capacity related to their self-management of their medical issue or engagement in recommended treatments.

- Identification of the interaction and impact of current mental health disorders and/or trauma on the gynaecological condition.
- The impact of the patient's past experiences in the medical system / previous clinicians and the effect of this on the patient's ability to engage in current services/clinics/treatments.
- Assessment of body image and function issues as a result of treatment for gynaecological health.

The duration of psychological medicine care for women referred by gynaecology and gynae-oncology should be aligned to:

- The referral question
- Care provided within a multi-disciplinary context
- Not beyond the timeframe of the gynaecology and gynae-oncology services provided, including referrals from the pain service.

Consideration of early referral to community or other appropriate care providers where assessment indicates longer term mental health interventions are recommended.

9. Treatment and specific interventions

9.1 Providing information and seeking informed consent

Staff must obtain informed consent for all patients for recommended treatments. This entails providing full information about available pharmacological and psychological treatments to the patient – and where appropriate, to their partner, carer, or family member.

Discuss all evidence-based available treatment options, including:

- Potential benefits of psychological interventions and pharmacological treatment
- Possible consequences of no treatment
- Possible harms associated with treatment
- Other treatment options
- What might happen if treatment is changed or stopped, particularly if pharmacological treatments are stopped abruptly
- Any individual factors relating to treatment options
- Manual informed delivery of evidence based psychological interventions is recommended above eclectic or unstructured interventions

A patient is considered to be legally competent to provide informed consent for treatment if they can:

- Understand a treatment option
- Are able to deliberate regarding its risks and benefits
- Form a decision based on this deliberation
- Communicate their decision adequately

For further information please see 'Chapter 2: Decision making capacity and informed consent' [Clinicians' Practice Guide to the Mental Health Act 2014](#)



9.2 Psychological treatment

The following interventions are available as options for the treatment for women treated at WNHS outside of this^{1,2,3,4}: Note the evidence base is based on fidelity to the intervention rather than selection of elements of an intervention.

Intervention and National Health and Medical Research Council (NHMRC) level of evidence	Target group/diagnostic criteria and indicated treatment plan	Who, timeframes for treatment	Target outcomes
Facilitated self-help, Psychoeducation, Online Cognitive Behaviour Therapy Directive counselling – including supportive listening, problem-solving and goal setting	Women with persistent subthreshold depression or anxiety symptoms, or mild to moderate depression Adjustment Disorder	As Per Referral Flow Chart refer for community support and management	
Individual structured psychological interventions which could include: Cognitive behavioural therapy Interpersonal psychotherapy Problem solving therapy Short term Psychodynamic psychotherapy	Women with mild to moderate depression; moderate to severe depression; or social anxiety disorder	Adult 6-18 sessions dependent on severity of symptoms	Resolution of presenting symptoms
Individual structured psychological interventions which could include Cognitive Behaviour Therapy (trauma focused)	Women with a current diagnosis of post-traumatic stress disorder (PTSD)	Adult	Resolution of presenting symptoms
Cognitive behavioural therapy for OCD (Exposure and Response Prevention -ERP)	Obsessive Compulsive Disorder	Adult 12-20 sessions	Resolution of presenting symptoms

Individual structured psychological interventions which could include Cognitive Behaviour Therapy for BPAD Interpersonal Psychotherapy	Bipolar Disorder	Adult 6-18 sessions	Resolution of presenting symptoms
Individual structured psychological interventions which could include Cognitive Behaviour Therapy for Psychosis and Schizophrenia	Psychosis Schizophrenia	Adult	Resolution of presenting symptoms
Provision of, or referral for, individual mother–infant relationship interventions which may focus parenting skills and/or the quality of the dyadic relationship, relevant in the presence of adequate training in such work	Parenting – including concerns in the domains of mother infant relationship, parenting stress, parenting behaviours/skills	Perinatal period	Resolution of presenting symptoms



Eating disorders

The treatment of eating disorders during pregnancy is outlined in the [Eating Disorders in Pregnancy](#) guideline. For psychological interventions:

- Psychological Medicine provide wrap-around care in consultation with medical treatment.
- Aim for early referral for rapid intervention.
- For sub-threshold diagnoses, or resolved but 'at-risk of relapse' during perinatal period, Psychological Medicine should offer assessment and/or consultation liaison-type service (i.e. educate patient regarding treatment pathways and options with primary care).
- For adult (non-pregnant) guidance on Eating Disorders refer to [WA Eating Disorders Outreach and Consultation Service](#).

Severe Mental Illness

The following psychological interventions are recommended for the treatment of Severe Mental Illness (SMI) (psychotic disorders and bipolar disorder) in pregnancy or the postnatal period:

- Psychoeducation and supportive, structured individual and cognitive behavioural therapy for women with severe mental illness; including bipolar disorder, psychosis and schizophrenia.

Psychological interventions should be offered to pregnant women with a diagnosed severe mental illness who are at risk of relapse particularly arising from:

- Stress associated with pregnancy or the postnatal period
- A change or cessation of medication.

Antenatal women with SMI are managed within CAMI. Women with SMI referred by Gynaecology and Gynae-oncology will be managed by Psychological Medicine in conjunction with their community treatment, private or primary care clinician.

Borderline Personality Disorder

The following interventions may be required for women with Borderline Personality Disorder (BPD) in the perinatal period:

- Psychological approaches to manage emotional dysregulation, self-harm and preparation for pregnancy and early parenthood (while specialist psychological treatments take effect).

Due to the nature of the Psychological Medicine service, there are no specific treatment programs offered for BPD, however behaviour management principles, therapeutic

interventions and support for the client is offered drawing on Dialectical Behaviour Therapy strategies to address mental health co-morbidities.

For an outline of evidence base outside of perinatal period please refer to the NHMRC [Clinical Practice Guideline for the Management of Borderline Personality Disorder](#), as well as the Mental Health Network [Statewide Model of Care for Personality Disorders](#).

9.3 Pharmacological treatment

General principles for use of pharmacological treatments

9.3.1 Gynaecology and Gynae-Oncology

For pharmacological treatment of non-pregnant or lactating women see specific guidelines for mental disorders (Royal Australian and New Zealand College of Psychiatrists (RANZCP) and/or NHMRC).

9.3.2 Perinatal Mental Health

For perinatal mental health and prescription of psychotropic medication in pregnancy, the quality of evidence on harm to the fetus is mostly of low to very low quality due to methodological limitations; the greatest being reliance on observational studies, as randomised controlled trials cannot be conducted in pregnant women on ethical grounds.

When discussing risks from pharmacological treatment options with the patient, balance treatment decisions and any benefits of treatment against the risk of untreated mental disorders and relapse (on both maternal and fetus/infant/child outcomes) if treatment is ceased. If a medication is ceased, this needs to be done gradually and under supervision from the prescriber.

The use of psychotropic medication in pregnancy includes risks to mother and the fetus/infant. Discuss the following known risks associated with agents or classes of agent with the patient, whether commencing or continuing medication:

- Teratogenicity – the potential to impact the development of the embryo or fetus.
- Increased risks of specific pregnancy complications.
- Increased risks of specific neonatal complications.
- Impacts on longer-term developmental outcomes for children exposed in-utero (developmental teratogenicity).

When considering treatment options, choose the agent with the lowest risk profile for the patient as well as the fetus/baby. This should:

- Consider the patient's previous response to medications.
- Use the lowest effective dose; but avoid ineffective treatment through sub-therapeutic dosing.
- Consist of a single prescribed agent, where possible.

Note that all psychotropic agents cross the placenta and all are excreted into breastmilk although the quantity varies between agents.

Consider changes in maternal physiology – gastric emptying, volume of distribution, renal clearance, decreased drug-binding capacity, and hepatic metabolism – that impact on pharmacokinetics and pharmacodynamics of psychotropic medication in pregnancy.

Preconception

Where relevant, consider pre-pregnancy modification of treatment to minimise teratogenic risk and to ensure optimal pre-pregnancy physical and mental health.

If treatment is to be altered, clinicians are to ensure a discussion on adequate contraception with the patient, given the high rate of unplanned pregnancies.

Where possible, check for vitamin D deficiency; ensure pre-conception folate supplementation and rubella immunisation; as well as ensuring routine physical screening associated with specific psychotropic agents is up to date. Please note higher folate supplementation (i.e. 5mg daily) is recommended for those on specific mood stabilisers or other conditions associated with health related risk factors for neural tube defects.

Sodium Valproate is not recommended for women planning pregnancy or where pregnancy is possible, and so should be avoided in women of childbearing age. **See ‘Associated risks and recommendations for the use of specific pharmacological treatments’** (below).

Monitoring during pregnancy

Therapeutic drug monitoring is necessary for women receiving medications such as lithium and lamotrigine.

Use the following recommended minimum monitoring schedule to ensure levels remain in the therapeutic range during pregnancy, additional testing of levels may be required depending on individual patient factors:^{3 4}

Lithium	Monthly serum lithium estimations throughout pregnancy
	Weekly monitoring of lithium levels in the third trimester
	Lithium levels prior to, and 24 hours after delivery
	Twice weekly lithium levels for the first 2 weeks postpartum
Lamotrigine	Therapeutic drug levels are usually only undertaken for patients with epilepsy however monthly drug level checks may be considered

³ Galbally et. al. (2010) ‘Management of antipsychotic and mood stabilizer medication in pregnancy: recommendations for antenatal care’, *Australian and New Zealand Journal of Psychiatry* 44 (2): doi: 10.3109/00048670903487217

⁴ Poels, Eline; Bijma, Hilmar; Galbally, Megan; Bergink, Veerle (2018) ‘Lithium during pregnancy and after delivery: a review’, *International Journal of Bipolar Disorders* 6 (26) <https://doi.org/10.1186/s40345-018-0135-7>

For monitoring of associated physical health parameters for anti-epileptics, lithium and anti-psychotics, see the following Pregnancy Monitoring Forms:³

- Anti-epileptics (MR215.11)
- Lithium Carbonate (MR215.12)
- Anti-psychotics (MR215.13)

Use of Tricyclic Antidepressants (TCA)

While there is a clear evidence base for the efficacy of TCA antidepressants there are also non-perinatal specific risks associated with use including the significant risk of harm with overdose. As such, careful consideration of both prescription and dispensing should be made and clear risk assessment and mitigation strategies documented when these agents are prescribed. In particular, weekly dispensing should be considered.

Specific delivery planning for women using lithium during pregnancy

- Include within the Perinatal Mental Health plan for patients on lithium during pregnancy (a) monitoring of lithium levels when admitted for delivery and (b) guidance to ensure adequate hydration during labour and delivery - with consideration of intravenous hydration if necessary.
- National Institute for Health and Care Excellence (NICE) and Centre of Perinatal Excellence (COPE) guidelines recommend ceasing or reducing lithium dose prior to the onset of delivery, however, there is currently *no evidence* that suggests this strategy decreases the risk of perinatal and infant complications, and this should be weighed against the risk of maternal relapse during a high-risk period.⁴
- Instead, carefully monitor lithium blood levels before and 24 hours after delivery, and adjust dosages as required.
- When possible, minimise or avoid administering the following medications:
 - Potential nephrotoxins that inhibit, damage, or destroy cells and/or tissues of the kidneys, including aminoglycosides and non-steroidal anti-inflammatory drugs.
 - Angiotensin-converting enzyme inhibitors (due to potential teratogenesis).
- Lower or withhold the lithium dose when renal throughput is compromised (such as in pre-eclampsia or haemorrhage).
- Due to the risks of neonatal complications, women should deliver where there are adequate facilities for neonatal resuscitation.
- Liaise with paediatric staff to ensure assessment and monitoring of the neonate after delivery for complications associated with *in utero* exposure, and cord blood lithium level should be taken at delivery.

- Continue to monitor maternal lithium twice weekly for the first 2 weeks postpartum. Unless there is a clinical indication for more frequent monitoring.
- Aim for therapeutic lithium levels after delivery and the first month postpartum for relapse prevention.
- Consider monitoring creatinine levels together with lithium levels in late pregnancy and during postpartum monitoring.

Associated risks and recommendations for the use of specific pharmacological treatments⁵

The following is for guidance purposes *only*, and should **not be considered comprehensive**. Also consider that evidence for all drug classes is mostly of ‘low’ to ‘very low’ quality, and recommendations summarised from COPE or National Institute for Health and Care Excellence (NICE) contained below comprise both consensus-based and evidence-based content.

It is **critical that clinicians stay up-to-date** with changing risk profiles and recommendations associated with the use of specific pharmacological treatments during pregnancy.

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
TCAs, Selective Serotonin Reuptake Inhibitors (SSRIs), Serotonin and norepinephrine reuptake inhibitors (SNRIs)	The evidence base for use of antidepressants in pregnancy and longer-term outcomes for children exposed is limited; though there is high quality RCT evidence of efficacy of antidepressants in the treatment of depression and anxiety in the general population – any increases in absolute risk of less-optimal birth outcomes/fetal harm are small. Increase in risk <i>is</i> associated with use of SSRI and SNRI Antidepressants for poor neonatal adaptation syndrome. Exposure to SSRI antidepressants may be associated with a small absolute risk for Persistent Pulmonary Hypertension of the Newborn	Compared to fetal exposure during pregnancy, exposure through breast milk is low The effects of exposure may be increased in preterm or otherwise unwell infants	Consider SSRIs as a first-line treatment for moderate to severe depression and/or anxiety in pregnant women; or for women with history of severe depression who present with mild depression in the perinatal period If a pregnant woman is taking a TCA, SSRI or SNRI for severe depression, take into account previous response to treatment, risk factors for miscarriage or preterm birth, stage of pregnancy, risk of relapse, risks and preferences associated with medication, and discuss the following options: – continue with the current medication – change medication if there is a drug that is effective with a lower risk of adverse effects – combine medication with a high-intensity psychological intervention – switch to a high-intensity psychological

⁵ This content is adapted from: National Institute for Health and Care Excellence, *Antenatal and postnatal mental health: clinical management and service guidance: Clinical guideline 192* (December 2014; last updated February 2020); and Austin, Marie-Paule, Highet, Nicole and the Expert Working Group (2017) *Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline* (Melbourne: Centre of Perinatal Excellence).

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
	The risk of miscarriage is unclear with the most recent meta-analysis not finding an association. Maternal use of SSRIs, SRIs and SNRIs at any time during pregnancy may be associated with increased risk of postpartum haemorrhage		intervention, if ceasing medication When considering the impact of antidepressants, including SSRIs to women who are breastfeeding, account for the infant's health and gestational age at birth
Benzo-diazepines and non-benzo-diazepine hypnotics	There is a lack of evidence regarding potential harms to the fetus associated with the use of non-benzodiazepine hypnotics (z-drugs) in pregnant women Benzodiazepines are <i>not</i> associated with increased risk of major malformation in the newborn, but there <i>is</i> evidence of: – increased risk for respiratory difficulty in the newborn following repeated late pregnancy exposure to long-acting benzodiazepines – increased risk for poor neonatal adaptation syndrome with use of benzodiazepines with short- or long-acting benzodiazepines	Most benzodiazepines are known to transfer into breastmilk in small amounts. Lethargy, sedation and poor suckling should be monitored. Shorter-acting benzodiazepines are preferred	Consider short-term use of benzodiazepines for treating moderate to severe symptoms of anxiety while awaiting onset of action of an antidepressant in pregnant or postnatal women Use caution in repeated prescription of long-acting benzodiazepines around the time of the birth Use caution in prescribing non-benzodiazepine hypnotics (z-drugs) to pregnant women for insomnia
Anti-psychotics	The evidence for use of anti-psychotics (as a class of drugs) in pregnancy and longer-term outcomes for children exposed is still very limited. Outside of pregnancy there is high quality	Most anti-psychotics are known to transfer into the breast milk in small amounts. Sedation, poor feeding and sleeping pattern changes should be	<i>See Monitoring During Pregnancy</i> It is important to appropriately treat severe mental illness in pregnancy If a pregnant woman develops mania or psychosis and is not taking psychotropic medication, offer an

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
	RCT evidence of efficacy of anti-psychotics in the general population	monitored.	anti-psychotic Offer an anti-psychotic as prophylactic medication if a woman with bipolar disorder becomes pregnant and is stopping lithium, or plans to breastfeed
Risperidone	First-trimester use of risperidone may be associated with an increase in absolute risk of major malformation and cardiac malformation. Use may be associated with increased risk of Gestational Diabetes Mellitus (GDM)	-	Early Glucose Tolerance Test (GTT)
Quetiapine	At higher doses quetiapine may be associated with increased risk of Gestational Diabetes Mellitus (GDM)	-	Early Glucose Tolerance Test (GTT)
Olanzapine	Weight gain is associated with most anti-psychotics, particularly olanzapine, in the general (non-pregnant) population Use may be associated with increased risk of Gestational Diabetes Mellitus (GDM)	-	Early Glucose Tolerance Test (GTT)
Clozapine	Clozapine crosses the placenta, which may increase the risk of agranulocytosis in the newborn Use may be associated with increased risk of Gestational Diabetes Mellitus (GDM)	The evidence on the safety of clozapine in breastfeeding women is limited as such this is not usually recommended.	The COPE guidelines recommend as a <i>consensus recommendation</i> to not initiate use of clozapine in pregnant women COPE guidelines recommend as a <i>practice point</i> the use of clozapine with caution in women who are breastfeeding and monitor the infant's white blood cell count weekly for the first six months of life and clinical consideration of monitoring beyond that.

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
			Given the complexity of managing treatment resistant psychotic disorders in pregnancy it is recommended the individual clinical presentation of a woman being considered for clozapine or continuing clozapine be discussed or reviewed with a senior perinatal psychiatrist with experience in this area. From this an individual treatment plan drawing on the latest evidence as well as the individual factors can be developed and discussed with the woman and as appropriate her partner/carer and other mental health clinical services involved in her care.
Anti-epileptics	There is risk of major malformation if conception occurs while a woman is using anticonvulsants Several anti-epileptics are folate antagonists	There is uncertainty about the passage into breastmilk of some anticonvulsants (e.g. lamotrigine) and adverse effects in the infant	<i>See Monitoring During Pregnancy</i> Only consider prescribing anti-epileptics (especially valproate) to women of child-bearing age if effective contraception is in place Use great caution in prescribing any anticonvulsants as mood stabilisers for pregnant women and seek specialist psychiatric consultation If anticonvulsants are prescribed to a woman who is breastfeeding, arrange close monitoring of the infant and specialist neonatologist consultation
Sodium valproate	Evidence of substantial increases in absolute risk of major malformation and cardiac malformation in the newborn Evidence of adverse cognitive outcomes in the child (e.g. increased risk of below average intelligence quotient [IQ])	Consider monitoring the infant's liver and platelet function and monitor for signs of jaundice or rash.	Do not prescribe sodium valproate to women who are planning a pregnancy or pregnant Once a decision to conceive is made, wean off valproate over 2–4 weeks, adding a high-dose folic acid (5 mg/day for the first trimester)

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
Carbamazepine	May be associated with an increased risk of major malformation in the newborn	Consider monitoring of liver enzymes and signs of jaundice, excessive drowsiness, vomiting and poor feeding	Use great caution in prescribing Carbamazepine for pregnant women
Lamotrigine	There is uncertainty about whether lamotrigine may have an association with increased risk of birth defects although the most recent update in 2021 did not show this**	If the choice is to breastfeed then ensure there is a clear plan for review of infant serum levels, full blood examination and liver function tests as well as monitoring for rash. Also ensure there is a plan to cease exposure (e.g. through temporarily ceasing breastfeeding and using formula feeding) if rash occurs and seek urgent medical review.	Use great caution in prescribing Lamotrigine for pregnant women Check lamotrigine levels frequently during pregnancy and into the postnatal period because they vary substantially at these times While levels are often not routine outside of pregnancy for bipolar in contrast to epilepsy management, a recommendation is to consider undertaking serum levels as early as possible in pregnancy as the 'target level' and then undertake levels every 4 weeks across pregnancy to ensure the levels that have resulted in being well outside of pregnancy are maintained given the likely requirement to increase dose for a woman to remain well and treated. Twice daily dosing where possible is also recommended. Dose increases are commonly required across pregnancy. After delivery ensure the dose returns gradually in steps to pre-pregnancy over the first 2 weeks postpartum and monitor for toxicity during this period.
Lithium	Recent large studies have confirmed the association between first trimester lithium	Potential for high passage of lithium into breastmilk and	See <i>Monitoring During Pregnancy</i> See <i>Specific delivery planning for women using</i>

Drug class	Risks in pregnancy*	Risks while breastfeeding*	Recommendations
	exposure and an increased risk of congenital malformations. Importantly, the risk estimates from these studies are lower than previously reported. Risk of Ebstein's Anomaly 0.05-0.1% Little is known about the association with preterm birth and birthweight, and developmental consequences of intrauterine exposure to lithium There is an association between lithium use and a range of pregnancy or delivery-related outcomes, but more research is needed	risk of infant toxicity as well as additional risks from lithium exposure for thyroid function, renal function and neurological side effects	<i>lithium during pregnancy</i> Breastfeeding while taking lithium is not recommended When lithium treatment must be continued, focus on ways to support mothers to feel confident and positive about parenting their baby, and to experience the closeness of the mother-infant bond without breastfeeding

Note: The [Centre of Perinatal Excellence \(COPE\) Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline](#) does not recommend the use of pharmacological treatments as the primary therapy for Borderline Personality Disorder, especially in pregnant women. This is because pharmacological treatments do not appear to be effective in altering the nature and course of Borderline Personality Disorder. However, they may be useful in the short-term in controlling more acute symptoms.

*See [Centre of Perinatal Excellence \(COPE\) Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline](#) (p.64) for quantitative overview of 'Increase in absolute risk of adverse outcomes associated with pharmacotherapy during pregnancy' per drug class. **<https://www.gov.uk/drug-safety-update/antiepileptic-drugs-in-pregnancy-updated-advice-following-comprehensive-safety-review#national-review-of-safety-data>

** See [Antiepileptic drugs in pregnancy: updated advice following comprehensive safety review by the Commission on Human Medicines](#)

9.4 Electroconvulsive Therapy (ECT)

General use in mental health

ECT is a safe and effective treatment considered a first-line option for:

- Severe depression associated with high suicide risk, catatonic signs, and/or psychotic symptoms whose physical health is at serious risk;
- Functional catatonia;
- Severe psychotic agitation (acute mania or psychosis).

ECT is also an effective treatment option for depression with response rates close to 70% in some studies. ECT may be a good option for those that have previously responded to ECT, or is unresponsive to multiple trials of antidepressant medications.

ECT in the antenatal period

Information on administering ECT in the antenatal period is within current [RANZCP ECT guidelines](#).

ECT in the postnatal period

For information on administering ECT in the Postnatal Period, see MBU Procedure Manual.

9.5 Complementary therapies

A range of complementary therapies are evaluated in the [Centre of Perinatal Excellence \(COPE\) Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline](#):

Omega-3 fatty acids: Omega-3 fatty acid supplementation does not appear to improve depression symptoms.

St John's Wort: The evidence on potential harms to the fetus from St John's Wort is limited and uncertain, and use of this treatment during pregnancy is not recommended.

Ginkgo biloba: Potential harms to the fetus from Ginkgo biloba have not been researched, and use of this treatment during pregnancy is not recommended.

Acupuncture: There is very low quality evidence that depression-specific acupuncture is more effective than non-depression-specific acupuncture in improving response to treatment.

9.6 Perinatal Mental Health Plans

For Perinatal Mental Health consider the use of both a [Sensitive Care Plan \(MR006.02\)](#) and [Perinatal Mental Health Special Instruction Sheet \(MR006.01\)](#) prior to delivery.

10. Discharge Planning

The processes and criteria for treatment, support and discharge planning for mental health services at WNHS are outlined in the Chief Psychiatrist's Standards for Clinical Care: [Care Planning Standard](#).

Mental health clinicians should complete the SSCD for Mental Health Services [Mental Health Treatment, Support and Discharge Plan](#) for all patients receiving mental health treatment. A letter to the referrer, the patient's general practitioner and where appropriate child health nurse, the patient and other relevant clinicians should be undertaken for all patients discharged from Psychological Medicine.

In the MBU, the Treatment, Support and Discharge Plan must be authorised by the treating psychiatrist and is developed in collaboration with the patient, their support person, the MBU multidisciplinary team, and relevant community services/stakeholders. The assessment must be of sufficient depth and breadth to be able to inform treatment, care and recovery planning.

A care plan is holistic and considers, at a minimum, the following key points:

- Mental health status including relapse triggers and markers;
- Risk assessment;
- Psychotropic medications;
- Ongoing therapy recommendations;
- The patient's physical health status (including contraception/post birth obstetric follow-up);
- Any co-morbid drug/alcohol issues;
- The patient's living situation and social supports; and
- The wellbeing of the infant, partner, siblings and other family members or carers (this may include consideration of parenting support, childcare needs and attention to specific physical, developmental and mental health needs of the infant).

Discharge planning occurs as part of the continuum of care which commences at the time of admission to a mental health service.

MBU staff should refer to the MBU Procedure Manual for further details about discharge planning and discharge process, including information sharing and discharge to a Community Treatment Order.

Psychological Medicine staff should refer to the Psychological Medicine Procedure Manual for further details about discharge planning and discharge process.

11. Telehealth

Telehealth is currently offered by the Department of Psychological Medicine for women eligible for care by the service and unable to attend in person and deemed clinically

suitable for telehealth. It is also offered by MBU for assessment of suitability for admission when required.

For further information on telehealth, see:

[Statewide Telehealth Service](#)

[WA Country Health Service, Use of Telehealth for Mental Health Assessments and Examinations Policy](#)

[RANZCP, Professional Practice Standards and Guides for Telepsychiatry](#)

[American Psychiatric Association, Best Practices in Videoconferencing-Based Telemental Health](#)

12. Risk Management/Safety and Security of Staff

Employers have a duty of care under the [Occupational Safety and Health Act 1984](#) to provide and maintain a safe work environment where employees are not exposed to hazards and can work without risk of injury or harm to themselves and other staff members. Patient and visitor initiated aggression in the government health system is identified as a workplace hazard.

The [NMHS Stop the Violence Strategy 2020-2025](#) aims to provide the highest standards of consistency in preventing and managing incidents of aggression across all NMHS locations. Reporting incidents of Workplace Violence and Aggression (WVA) ensures the hospital implements appropriate strategies and directs resources to where they are required. A NMHS [WVA Incident Report Form](#) should be completed for any incident where staff feels threatened or unsafe.

Within WNHS managers are expected to implement strategies to prevent exposure to violence and aggression as well as manage incidents that occur through the completion of a [Workplace Violence Risk Assessment](#)

Please see relevant Procedures in Psychological Medicine and MBU.

Principles in responding to requests for post incident trauma debriefing

1. Post-trauma interventions **should not** include any form of debriefing such as Psychological Debriefing.
2. Ideally post-trauma support such as in the form of Psychological First Aid should be provided by managers and supported across the organisation.

13. Documentation

Mental health care is dependent upon good clinical documentation. The Mother and Baby Unit and Psychological Medicine use the [State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#) for the provision of referral and management for women and perinatal mental health. This is mandated by the WA Department of Health Policy Framework [MP 0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#) and is supported by the [Statewide Standardised Clinical Documentation for Mental Health Services Procedure](#).

In addition, the [Triage to Discharge](#) document provides a mental health framework to ensure patient treatment, support and discharge plan is accurately communicated over time.

The following SSCD documents are used in WNHS in paper based medical record form.

- [Mental Health Triage \(SMHMR900\)](#)
- [Mental Health Treatment, Support and Discharge Plan \(SMHMR907\)](#)
- [Adult Mental Health Physical Examination \(SMHMR903\)](#)
- [Adult Mental Health Risk Assessment and Management Plan \(SMHMR905\)](#)
- [Adult Mental Health Assessment \(SMHMR902\)](#)
- [Adult Mental Health Physical Appearance \(SMHMR904\)](#)
- [Adult Mental Health Care Transfer Summary \(SMHMR916\)](#)

The following medical record forms are used to support psychological referral and management in WNHS:

- MBU Referral Form
- WNHS Mothercrafting Assessment
- Mother and Baby Unit, admission procedure checklist
- Baby Assessment
- Mother and Baby Unit discharge plan and checklist
- Mother and Baby Unit consent for exchange of information participation in research & QI
- Mother and Baby Unit consent for leave (MR208.12)
- Specials and Clinical Observation Report (MR285.15)
- WHCCU mental health permission to communicate confidential information and identification of personal support person (MR208.11)
- Referral Form: Department of Psychological Medicine (MR208.13)
- Youth - Mental Health Assessment (MR086)
- Sensitive Care Plan – Mental Health (MR215.10)
- Antenatal Risk Questionnaire (ANRQ) (MR312.01)
- Edinburgh Postnatal Depression Scale (EPDS): Post-natal
- Edinburgh Postnatal Depression Scale (EPDS): Antenatal
- Pregnancy Monitoring Anti-epileptics (MR215.11)

- Pregnancy Monitoring Lithium Carbonate (MR215.12)
- Pregnancy Monitoring Anti-psychotics (MR215.13)
- Perinatal Mental Health Plan: Special Instruction Sheet (MR006)

Definitions and Abbreviations

ANRQ	Antenatal Risk Questionnaire
CAMI	Childbirth and Mental Illness Clinic
COPE	Centre of Perinatal Excellence
CPFS	Child Protection and Family Support
ECT	Electroconvulsive Therapy
EPDS	Edinburgh Postnatal Depression Scale
GDM	Gestational Diabetes Mellitus
GTT	Glucose Tolerance Test
KEMH	King Edward Memorial Hospital
MBU	Mother and Baby Unit
NICE	National Institute for Health and Care Excellence
NHMRC	National Health and Medical Research Council
NMHS	North Metropolitan Health Service
NOCC	National Outcome Casemix Collection
OCD	Obsessive Compulsive Disorder
RANZCP	The Royal Australian and New Zealand College of Psychiatrists
SMI	Severe Mental Illness
SSCD	State-wide Standardised Clinical Documentation
SSRI	Selective Serotonin Reuptake Inhibitors
TCA	Tricyclic Antidepressant

Trauma Informed Care and Practice (TICP)	“An approach which recognises and acknowledges trauma and its prevalence, alongside awareness and sensitivity to its dynamics, in all aspects of service delivery” (Mental Health Coordinating Council, 2017)
WNHS	Women and Newborn Health Service
WVA	Workplace Violence and Aggression

References and resources

[Centre of Perinatal Excellence \(COPE\) Mental Health Care in the Perinatal Period: Australian Clinical Practice Guideline](#)

[National Institute for Health and Care Excellence Antenatal and postnatal mental health: clinical management and service guidance](#)

[State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)

[Statewide Telehealth Service](#)

[WA Country Health Service, Use of Telehealth for Mental Health Assessments and Examinations Policy](#)

[RANZCP Position Statement 57 Perinatal Mental Health Services](#)

[RANZCP, Professional Practice Standards and Guides for Telepsychiatry](#)

[American Psychiatric Association, Best Practices in Videoconferencing-Based Telemental Health](#)

[Adult NOCC Rater Clinical utility Training Manual](#)

[Mental Health National Outcomes and Casemix Collection - Technical Specification of State and Territory Reporting Requirements Verion 2.02.](#)

[Charter of Mental Health Care Principles](#)

[Mental Health Coordinating Council](#)

Related policies

[Mental Health Act 2014](#)

[Children and Community Services Act 2004](#)

[Occupational Safety and Health Act 1984](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

Related policies

[WA Department of Health Guidelines for Protecting Children 2015](#)

[Child Protection and Child Sexual Abuse \(Mandatory Reporting\) - WNHS Policy](#)

[WA Department Of Health Clinicians' Practice Guide to the Mental Health Act 2014](#)

[OD 0526/14 - State-wide Standardised Clinical Documentation SSCD for Mental Health Services \(PDF 141KB\)](#)

[DOH MP 0065/17 Aboriginal Cultural eLearning Policy](#)

Related WNHS procedures and guidelines

[Management of Mental Health Emergency - Clinical Practice Guideline](#)

[Child Protection - Social Work Clinical Practice Guideline](#)

[Eating Disorders in Pregnancy Clinical Practice Guideline](#)

Useful resources (including related forms)

[Department of Psychological Medicine - Referral Form](#)

[MBU Referral Form](#)

[Mental Health Assessment Form](#)

[Mental Health Treatment, Support and Discharge Plan](#)

[SSCD Mental Health Services Risk Assessment and Management Plan](#)

[Edinburgh Postnatal Depression Scale](#)

[Antenatal Risk Questionnaire](#)

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The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this document (ISD Number - 530).

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